

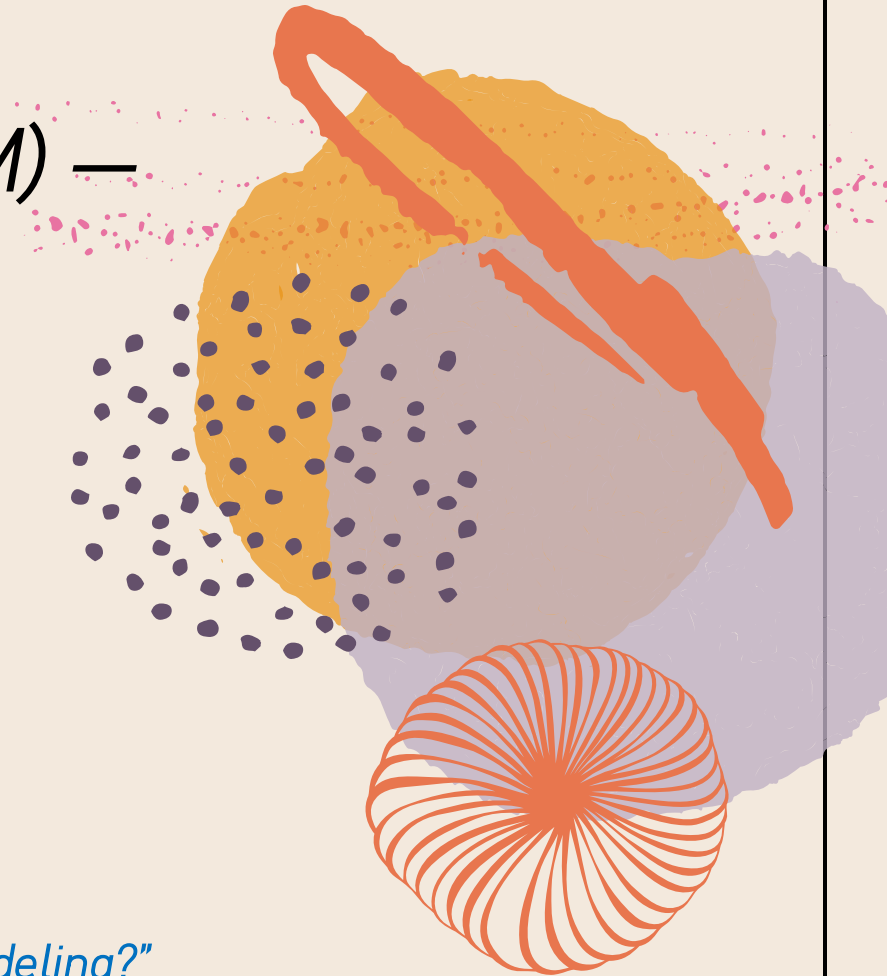
Wisconsin Breast Cancer Diagnosis (B vs M) —

Multivariate Classification & Interpretation

SEP767 – Final Project

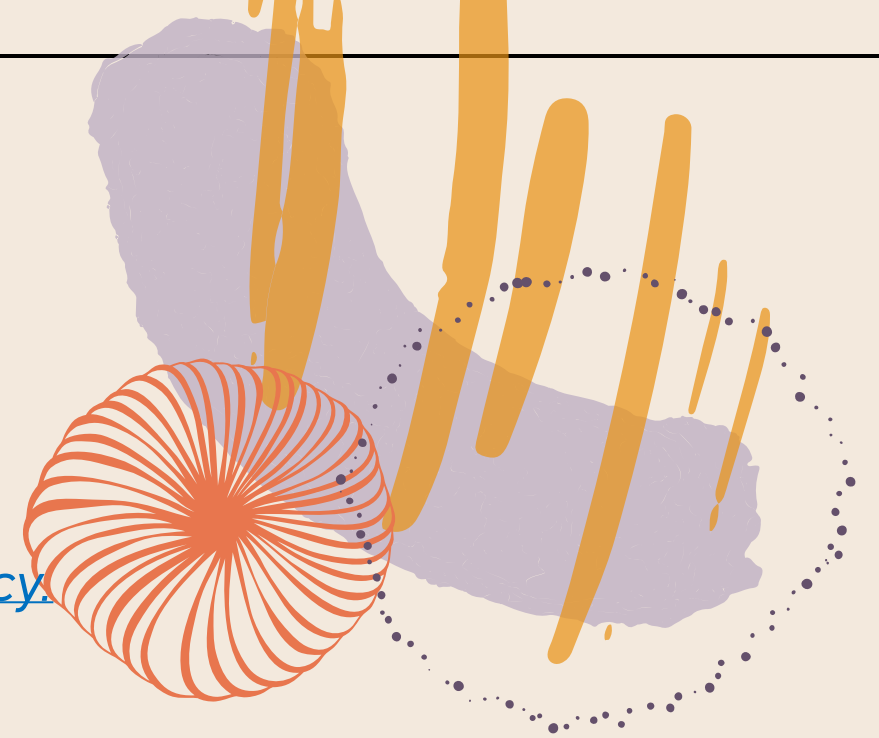
Andi Dong

“Can unsupervised compression compete with supervised latent modeling?”



Title & Problem Context

- *Significance:* Accurate and interpretable early screening models can support clinical decision-making by reducing missed malignant cases and improving diagnostic consistency.
- *Dataset:* Wisconsin Diagnostic dataset (from Kaggle)
- *Goal:* high sensitivity for malignant detection + interpretable features
- *Overview of methods:* PCA+RF baseline (CV) + PLS-DA (proper validation) + VIP + multiblock interpretation



PIPELINE

Data

Preprocess

Models

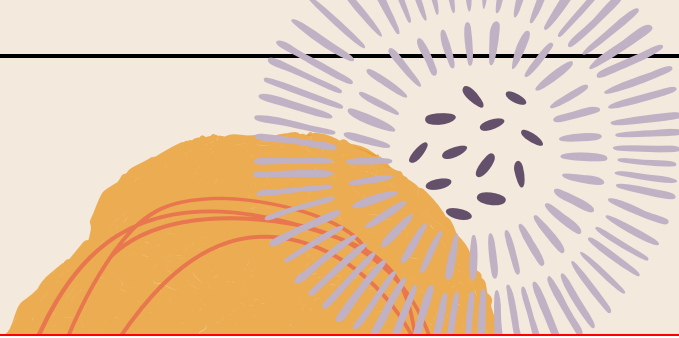
Evaluation

Interpretation

Related Work: PCA and PLS-DA in Biomedical Classification

- PCA-based dimensionality reduction has been widely adopted in cancer-related high-dimensional datasets to reduce redundancy and improve classification robustness. Studies on microarray cancer data demonstrate that [PCA significantly improves classification accuracy by addressing the imbalance between feature dimensionality and sample size](#) (Adiwijaya et al., 2018)
- Multivariate statistical methods, including PCA and PLS-based approaches, have shown strong performance in breast cancer diagnosis. Compared to univariate analysis, [multivariate models achieve higher accuracy and better class separability](#) in complex biomedical data (Kim et al., 2009)
- PLS-DA, as a supervised multivariate technique, has been extensively applied in biomedical classification tasks. Comparative studies indicate that [PLS-DA achieves high accuracy, sensitivity, and specificity](#) (typically above 90%) in high-dimensional and highly correlated biomedical datasets (Lasalvia et al., 2022)





Database Description

- a) radius (mean of distances from center to points on the perimeter)
- b) texture (standard deviation of gray-scale values)
- c) perimeter
- d) area
- e) smoothness (local variation in radius lengths)
- f) compactness ($\text{perimeter}^2 / \text{area} - 1.0$)
- g) concavity (severity of concave portions of the contour)
- h) concave points (number of concave portions of the contour)
- i) symmetry
- j) fractal dimension ("coastline approximation" - 1)

The mean, standard error and "worst" or largest (mean of the three largest values) of these features were computed for each image,

- Samples: 569 cell nucleus
- Features: 30 numeric features after dropping id/empty column

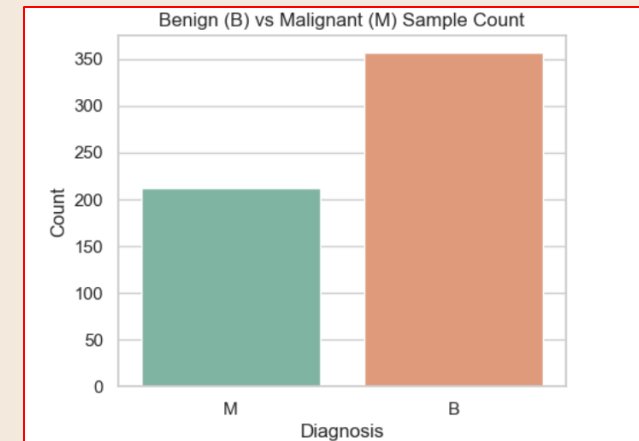
Step 1 data preprocessing! $X_{\text{scaled}} : (569, 30)$ $y : (569,)$

- Labels: B=357, M=212 (Diagnosis (M = malignant, B = benign))
- Label encoding: B→0, M→1

```
diagnosis
B      357
M      212
Name: count, dtype: int64
```

映射后的标签统计:

```
diagnosis_num
0      357
1      212
Name: count, dtype: int64
```





Objectives

- Build a baseline classifier using PCA + Random Forest
- Build a rigorously validated PLS-DA model (train/CV, test threshold, validation final)
- Provide interpretability via VIP + multiblock PCA (mean/se/worst)

Preprocessing & PCA --- Methodology

- Drop non-informative columns (id, empty)
- Encode label, split X/y
- Standardize features (z-score) before PCA/PLS
- PCA for visualization + dimension reduction

```
# delete id and Unnamed: 32 if exists  
cols_to_drop = []
```

```
if "id" in data.columns:  
    cols_to_drop.append("id")  
if "Unnamed: 32" in data.columns:  
    cols_to_drop.append("Unnamed: 32")
```

```
data = data.drop(columns=cols_to_drop)
```

```
# ===== split X y =====  
# feature columns: all numerical features except diagnosis and diagnosis_num  
feature_cols = [col for col in data.columns  
                 if col not in ["diagnosis", "diagnosis_num"]]
```

```
X = data[feature_cols]  
y = data["diagnosis_num"]
```

```
# ===== standardized features =====  
scaler = StandardScaler()
```

```
# 注意: fit_transform 返回的是 numpy 数组  
X_scaled_array = scaler.fit_transform(X)
```

```
# ===== Run PCA =====  
pca = PCA()  
pca.fit(X_scaled_array)
```

```
# 获取PCA后的样本坐标 (scores)  
X_pca = pca.transform(X_scaled_array)
```



Two modeling tracks --- Methodology

- Track A: PCA + RF (baseline)
 - Use first PCs as inputs (baseline used PC1-PC3)
 - Report confusion matrix and accuracy
 - Then select PC number via Pipeline + 5-fold CV, choose best PCs (best=5)

```
# ===== 划分训练集和测试集 =====  
# test_size=0.2 表示 20% 作测试, 随机数种子设置为 42, 保证结果可复现  
X_train, X_test, y_train, y_test = train_test_split(  
    X_pca_3, y, test_size=0.2, random_state=42  
)  
  
# ===== 定义 Random Forest 模型 =====  
rf = RandomForestClassifier(  
    n_estimators=200,      # 决策树数量  
    max_depth=None,       # 不限制树深度  
    random_state=42  
)  
  
# ===== 训练模型 =====  
rf.fit(X_train, y_train)
```

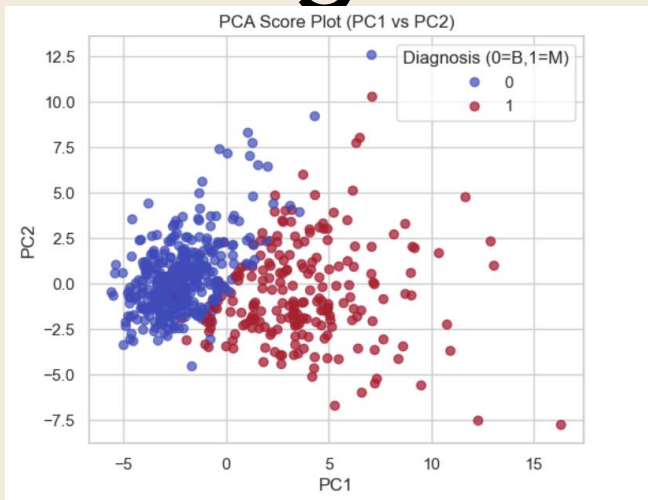
- Track B: PLS-DA (golden rules)
 - Train (CV select LV by ROC-AUC)
 - Test (tune threshold using Youden's J)
 - Validation (final metrics only)

```
scaler = StandardScaler()  
X_train_s = scaler.fit_transform(X_train)  
X_test_s = scaler.transform(X_test)  
  
pls = PLSRegression(n_components=best_lv)  
pls.fit(X_train_s, y_train)  
  
y_test_score = pls.predict(X_test_s).ravel()
```

My using rules!

- Stratified Cross-Validation: StratifiedKFold was used due to the imbalanced sample sizes between benign and malignant classes.
- Decision Threshold Selection: The decision threshold on the testing set was determined using Youden's J, rather than a fixed threshold of 0.5.
- Final Model Training: For final validation, the model was retrained using the combined training and testing datasets.

PCA insight --- Results



first two PC loadings:

	PC1	PC2
radius_mean	0.218902	-0.233857
texture_mean	0.103725	-0.059706
perimeter_mean	0.227537	-0.215181
area_mean	0.220995	-0.231077
smoothness_mean	0.142590	0.186113

- a clear separation trend between benign and malignant tumors along PC1
- PC1 is dominated by size-related features (radius, perimeter, area)

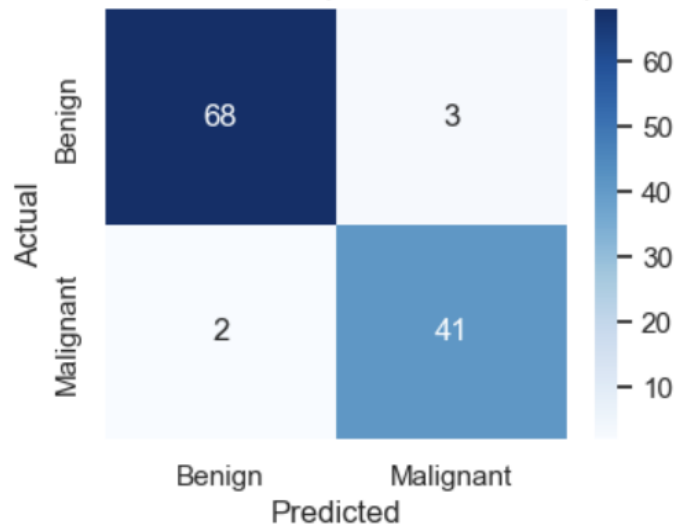
PCA+RF baseline + CV selection --- Results

Using 3 components as a base line

```
accuracy = accuracy_score(y_test, y_pred)
print("Random Forest Accuracy (using PC1-PC3): {:.4f}".format(accuracy))
```

Random Forest Accuracy (using PC1-PC3): 0.9561

Confusion Matrix (RF with PC1-PC3)

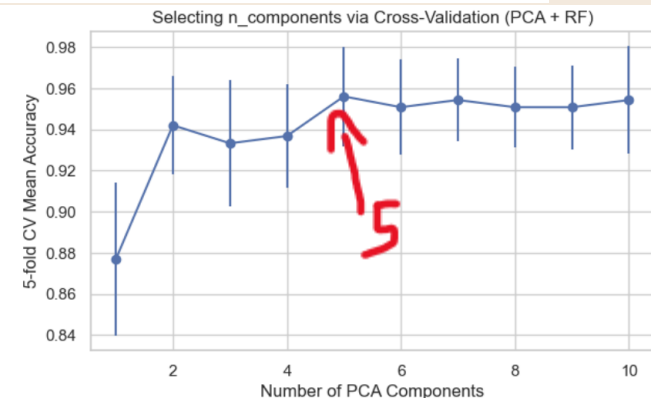


Using Cross-Validation to find the optimum # of components

n_components	cv_mean_accuracy	cv_std
0	1	0.876929 0.037406
1	2	0.942059 0.023865
2	3	0.933271 0.030626
3	4	0.936796 0.024989
4	5	0.956094 0.024149
5	6	0.950831 0.023229
6	7	0.954339 0.020264
7	8	0.950831 0.019639
8	9	0.950815 0.020417
9	10	0.954324 0.026233

PC # = 5

最佳PC数量 (CV mean最高) : 5
对应CV accuracy: 0.9561 ± 0.0241

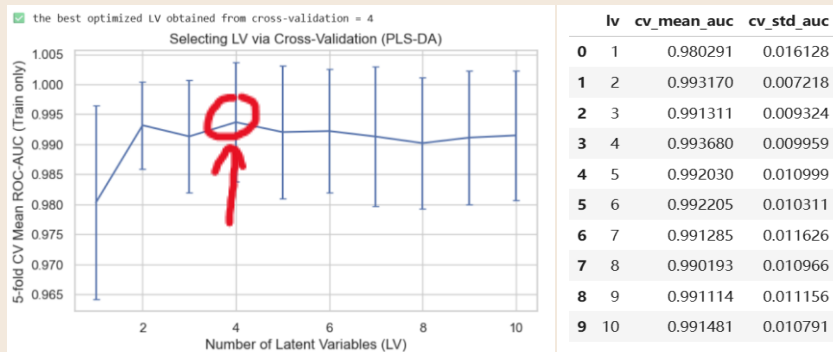


PLS-DA validation performance --- Results

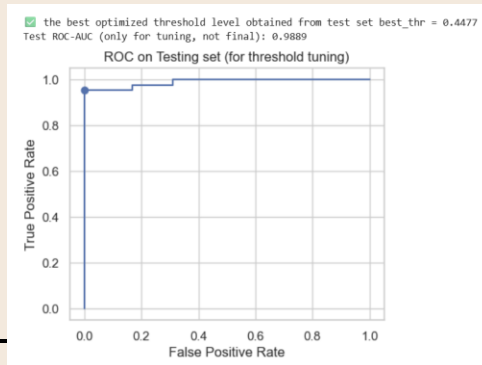
60/20/20 : Train 60%, Test 20%, Validation 20%

Train: (341, 30) Test: (114, 30) Validation: (114, 30)
Train class ratio: {0: 0.628, 1: 0.372}
Test class ratio: {0: 0.623, 1: 0.377}
Val class ratio: {0: 0.632, 1: 0.368}

The best LV obtained from CV = 4 based on AUC



The best threshold obtained from test set =0.4477



Results from the final validation set

===== FINAL (Validation Set Only) =====

Confusion Matrix:

```
[[72  0]
 [ 2 40]]
```

Accuracy = 0.9825

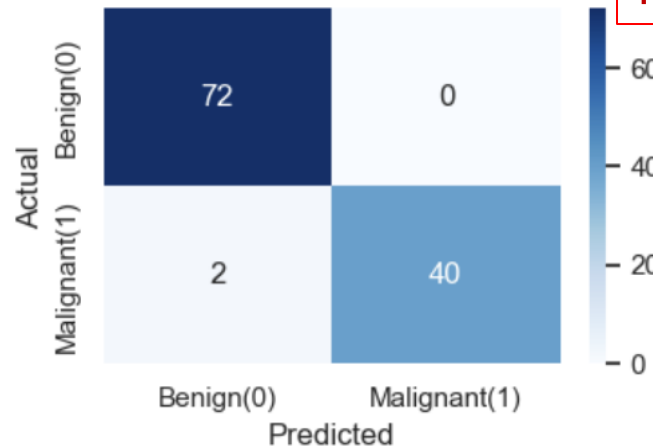
Sensitivity = 0.9524 (恶性检出率, 越高越少漏检)

Specificity = 1.0000 (良性识别率)

ROC-AUC = 0.9944

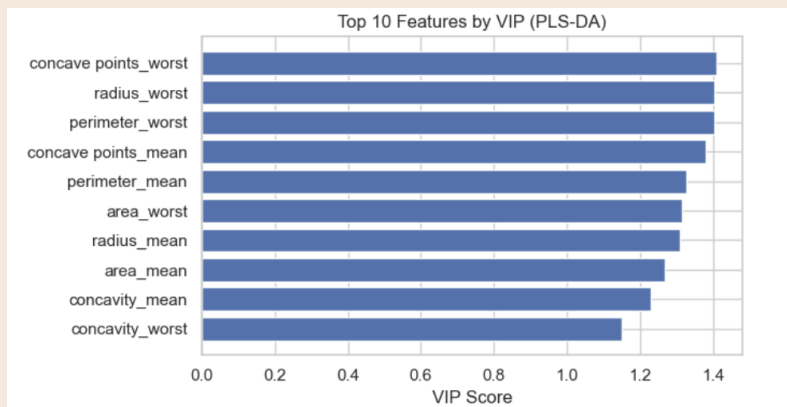
PR-AUC = 0.9924

Confusion Matrix (Final - Validation)



Much Better Result!

Interpretation (VIP + “what drives malignancy”) (Discussion)



Top 10 VIP features:

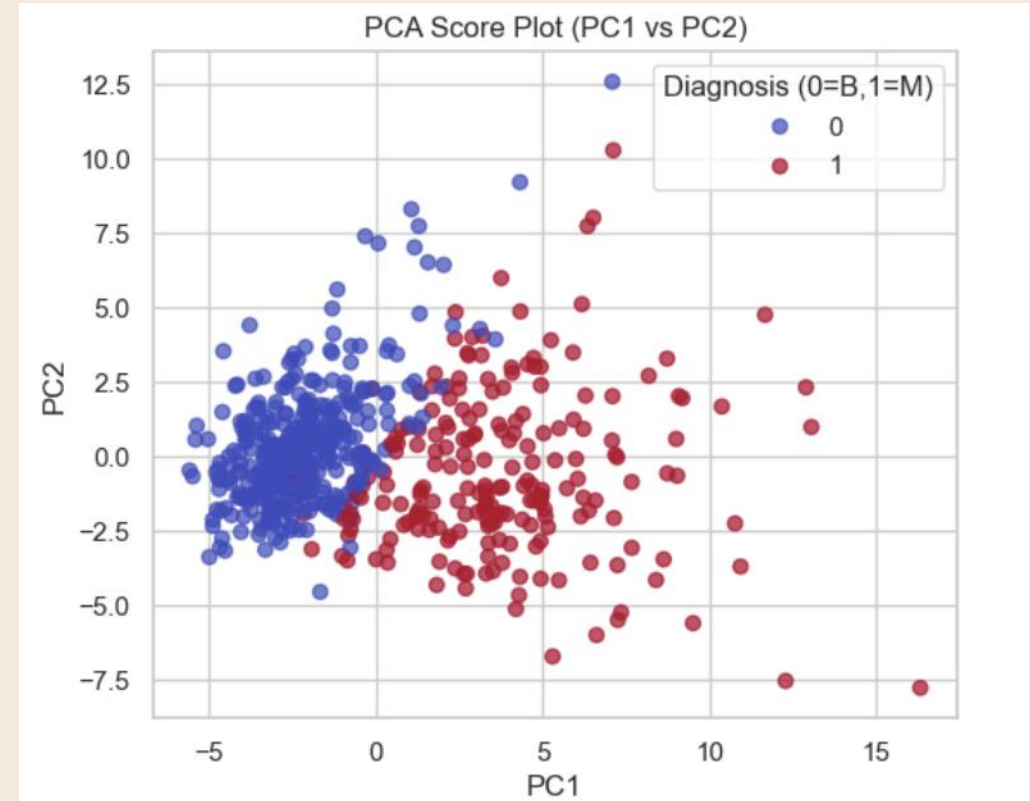
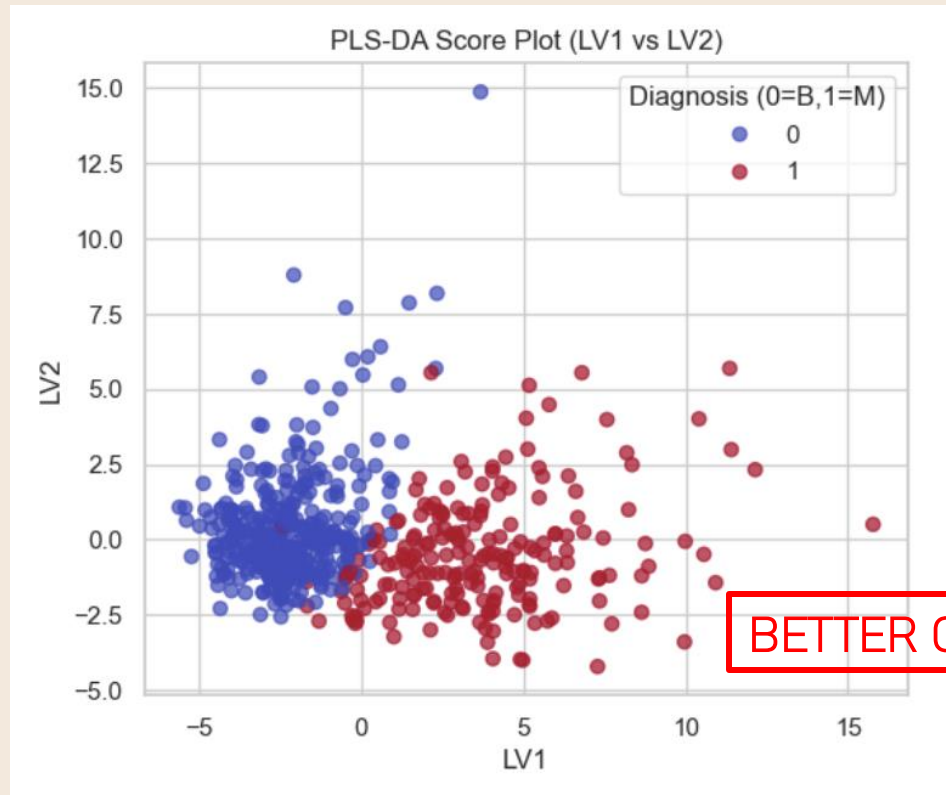
	feature	VIP
27	concave points_worst	1.409697
20	radius_worst	1.404007
22	perimeter_worst	1.403902
7	concave points_mean	1.380496
2	perimeter_mean	1.327158
23	area_worst	1.315342
0	radius_mean	1.310553
3	area_mean	1.267173
6	concavity_mean	1.230833
26	concavity_worst	1.149302

- The worst concave points feature is the most discriminative variable
- Highlighting the importance of extreme morphological irregularity in malignant tumors.
- The discrimination between benign and malignant tumors is mainly driven by extreme geometric irregularities and worst-case morphology

PLS-DA vs PCA under # of Components = 2(use for comparison)

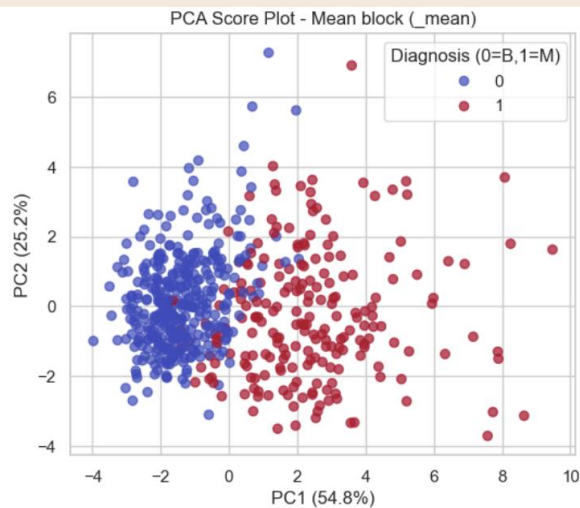
PLS-DA(trained with final validation set using full X)

PCA(using full X)

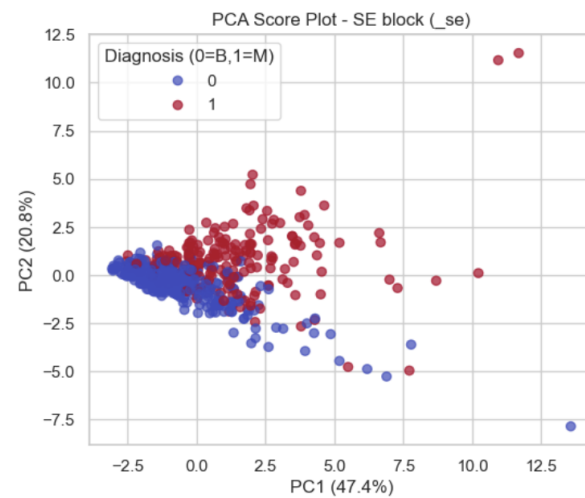


Multiblock Analysis (Discussion Extension)

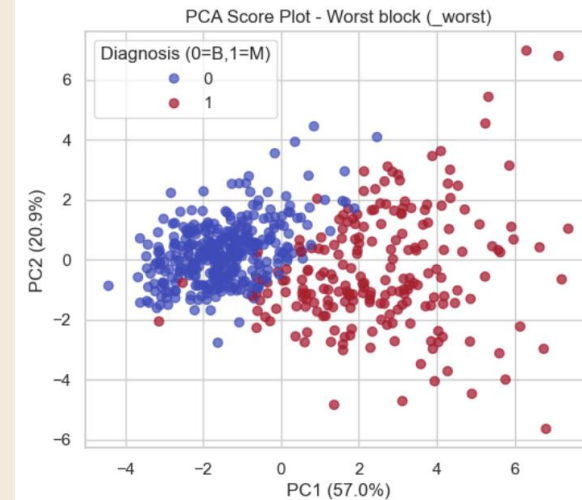
- Block 1 --> `_mean(10)` ✓ Block with greatest separation from PC1?
- Block 2 --> `_se(10)` ✓ Variables with greatest loadings in the block?
- **Block 3 --> `_worst(10 vars)`** Block sizes: {'Mean block (`_mean`)': 10, 'SE block (`_se`)': 10, 'Worst block (`_worst`)': 10}



```
=== Mean block (_mean) ===  
Top PC1 loadings:  
concave points_mean    0.418038  
concavity_mean         0.395748  
perimeter_mean         0.376044  
compactness_mean       0.364442  
area_mean              0.364086  
dtype: float64  
Top PC2 loadings:  
fractal_dimension_mean  0.571768  
smoothness_mean        0.401962  
symmetry_mean          0.368301  
radius_mean            -0.313929  
area_mean              -0.304842  
dtype: float64
```



```
=== SE block (_se) ===  
Top PC1 loadings:  
concave points_se      0.385743  
compactness_se        0.374799  
perimeter_se          0.357481  
concavity_se          0.355553  
radius_se             0.345592  
dtype: float64  
Top PC2 loadings:  
area_se               0.500211  
radius_se            0.440354  
perimeter_se         0.420303  
fractal_dimension_se -0.352502  
smoothness_se        -0.270953  
dtype: float64
```



```
=== Worst block (_worst) ===  
Top PC1 loadings:  
concave points_worst    0.397637  
concavity_worst        0.374742  
compactness_worst      0.364568  
perimeter_worst        0.348151  
radius_worst           0.335110  
dtype: float64  
Top PC2 loadings:  
fractal_dimension_worst 0.784956  
area_worst             -0.400000  
radius_worst           -0.403137  
perimeter_worst        -0.375518  
smoothness_worst       0.337870  
dtype: float64
```

The best
Separation

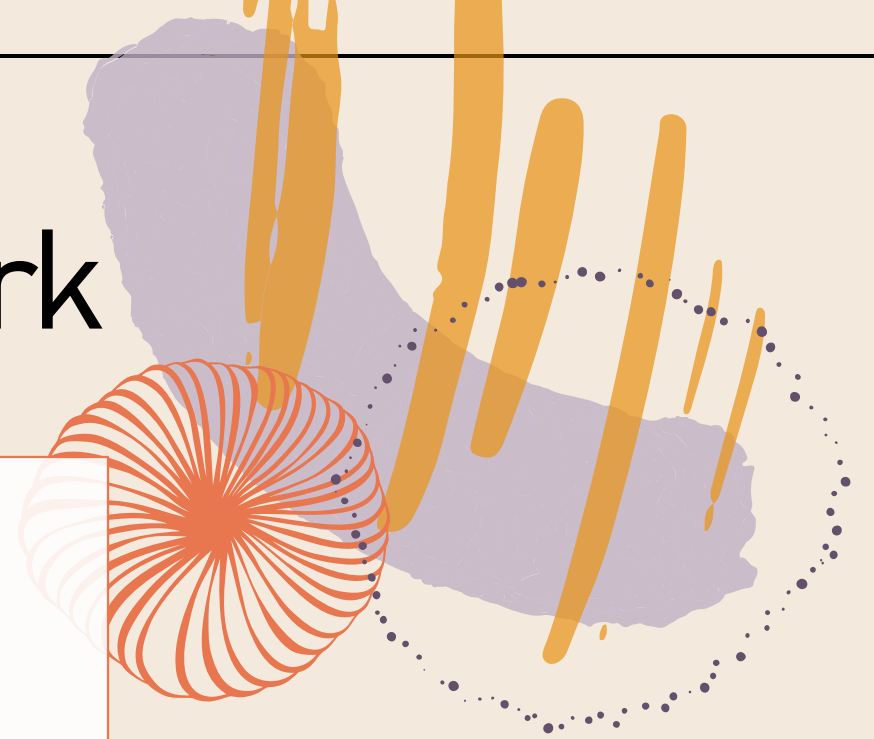
Conclusions & Future Work

Conclusion

- PCA reveals a clear separation trend along PC1; size/morphology features dominate.
- PCA+RF baseline achieved $\text{Acc} \approx 0.956$; CV suggested best PCs = 5.
- Properly validated PLS-DA achieved $\text{Acc} = 0.9825$, $\text{Sens} = 0.9524$, $\text{Spec} = 1.0000$ (Validation only).
- **VIP & multiblock consistently indicate worst-case morphology (e.g., concave points_worst) drives malignancy separation.**

Future Work

- Use nested CV and compare with alternative classifiers (logistic/SVM) on PCA or PLS scores.
- Optimize threshold for clinical preference (cost-sensitive, prioritize Sensitivity).
- External validation / larger dataset; add calibration and confidence intervals (bootstrap).





Reference

1. Adiwijaya, W. U., Lisnawati, E., Aditsania, A., & Kusumo, D. S. (2018). Dimensionality reduction using principal component analysis for cancer detection based on microarray data classification. *Journal of Computer Science*, 14(11), 1521-1530.
2. Lasalvia, M., Capozzi, V., & Perna, G. (2022). A Comparison of PCA-LDA and PLS-DA Techniques for Classification of Vibrational Spectra. *Applied Sciences*, 12(11), 5345. <https://doi.org/10.3390/app12115345>
3. Kim, Y., Koo, I., Jung, B. H., Chung, B. C., & Lee, D. (2009, November). Multivariate classification of urine metabolome profiles for breast cancer diagnosis. In *Proceedings of the third international workshop on Data and text mining in bioinformatics* (pp. 11-18).
4. [UCI Machine Learning]. ([Sep 25, 2016]). [Breast Cancer Wisconsin (Diagnostic) Data Set], [Version 2]. Retrieved [Dec 15, 2025] from [[Breast Cancer Wisconsin \(Diagnostic\) Data Set](#)].

Thank you!

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